

Pelvic exenteration

Pelvic exenteration is surgery to remove multiple organs in the pelvis. It is sometimes used to treat cancers in this area of the body.

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What is pelvic exenteration?

Pelvic exenteration is surgery to remove multiple organs in the pelvis. It is sometimes used to treat cancers in this area of the body.

The pelvis is the area of the body between the hip bones, in the lower part of the tummy (abdomen). It contains:

- the sex organs
- the bladder
- a section of the small bowel
- the lower end of the large bowel (colon, rectum and anus).

Pelvic exenteration can be used when cancer has:

- spread within the pelvis
- come back in the pelvis after other treatment.

Pelvic exenteration can cure cancer in some people. But it is a major operation. It is important to discuss the benefits and risks with your surgeon, before having it.

Some people have other cancer treatments before having pelvic exenteration surgery. This is called neo-adjuvant treatment. The aim is to try and shrink the cancer before the surgery.

Pelvic exenteration is sometimes called pelvic clearance.

Types of pelvic exenteration

There are different types of pelvic exenteration. The type you are likely to have will depend on your sex and where the cancer is.

We have more information about:

- [pelvic exenteration for men and people assigned male at birth](#)
- [pelvic exenteration for women and people assigned female at birth](#).

Deciding whether pelvic exenteration is right for you

Pelvic exenteration is only suitable for a small number of people. A team of health professionals will work together to plan the treatment they feel is best for you. This team is called a [multidisciplinary team \(MDT\)](#).

If pelvic exenteration may be suitable for you, you will be referred to a specialist centre to talk to a:

- surgeon, who has experience of this type of surgery
- specialist cancer nurse, who can give you information and emotional support.

They will talk with you about the following:

- Whether the operation is right for you
Your surgeon or specialist nurse can help you decide whether the operation is right for you. They will tell you what the operation involves. If your hospital allows it, you may find it helpful to take a family member or friend to the appointments.
- How you will cope with recovery
Recovery after pelvic exenteration can be difficult, both physically and emotionally. Your surgeon or specialist nurse can help you understand more about what recovery will be like. You will likely need support when you first get home. Your surgeon or nurse will talk to you about what support you have or may need.

It can be helpful to talk to someone who has had the operation. Your surgeon or specialist nurse may be able to arrange this for you. Or you may find someone who has had a similar surgery in our [Online Community](#).

Before pelvic exenteration

Pelvic exenteration is a major operation, so your doctors need to make sure you can cope with the surgery. If you have other health problems, it may be more difficult for you to recover from the operation. You may have heart and lung tests. This is to check your general health and fitness.

Getting ready for treatment

Your doctor or specialist nurse may give you information about prehabilitation (prehab). Prehab means getting you as ready for treatment as you can be. To help you prepare, your team may give you advice and support about:

- exercise

- healthy diet and weight
- stopping smoking
- reducing alcohol intake
- mental health and wellbeing.

This can help you get fitter for surgery and may help with your recovery. It is important to get advice from your doctor or nurse about exercise and making changes to your diet.

If you have been having problems with eating, or have lost weight, you may be referred to a dietitian. They can help and support you with your diet and nutrition before and after the surgery.

We have more information about [preparing for surgery](#).

Having tests

Your doctors will also do tests to see whether the cancer has spread. They will discuss what tests they think you should have before having the surgery.

Blood tests



[Blood tests](#) check how well your kidneys and liver are working and show the number of blood cells in the blood.

CT scan



A [CT scan](#) takes a series of x-rays, which build up a detailed picture of the inside of the body.

PET-CT scan



A [PET-CT scan](#) is a combination of a CT scan and a PET scan. Combining the scans gives more detailed information about the part of the body being scanned.

MRI scan



An [MRI scan](#) uses magnetism to build up a detailed picture of areas of your body.

Examination under anaesthetic (EUA)



You may have an EUA so your surgeon can find out the extent of the cancer. They may take small samples of tissue (biopsies). This may be an [EUA of the anal canal and rectum](#) or an [EUA for gynaecological cancers](#).

Cystoscopy



A [cystoscopy](#) (also called a flexible cystoscopy) is a test that looks at the urethra and the inside of the bladder. To do this your doctor or nurse uses a cystoscope. A cystoscope is a thin tube with a camera and light on the end.

Sigmoidoscopy



A [sigmoidoscopy](#) is a test that looks at the inside of the rectum and the part of the colon closest to the rectum (the sigmoid colon). You can usually have it as an outpatient.

You may be given medication to take before this test to help to empty your bowel.

A sigmoidoscope is a tube with a light and camera on the end. You lie curled on your left side and a doctor or nurse passes the tube into your back passage. A small amount of air is pumped into the bowel to make it easier to see inside it.

Colonoscopy



A [colonoscopy](#) is a way of examining the lining of the bowel from the inside. A doctor or nurse passes a thin, flexible tube (colonoscope) into your back passage. The tube has a tiny light and camera on the end to show any abnormal areas.

Biopsies



A [biopsy](#) is when doctors remove a small piece of tissue or a sample of cells from an area of the body. This is then sent to be checked under a microscope. You may have biopsies taken to help doctors understand the cancer. Biopsies may also be taken to check that the cancer has not spread outside the pelvis.

A biopsy is usually done by putting a fine needle into the area to be tested. The needle can take a sample of cells. This is called a fine needle biopsy (FNB) or fine needle aspiration (FNA).

You may have biopsies taken during an EUA, sigmoidoscopy, colonoscopy or cystoscopy.

Having pelvic exenteration

If you and your doctors agree pelvic exenteration is the right treatment for you, you will go to a specialist centre for the operation. You will be asked to give your permission ([consent](#)) before you have surgery.

The operation is usually done by 2 or more surgeons working together. These could include a:

- urologist – a doctor who treats problems with the kidneys, bladder and male reproductive system

- colorectal surgeon – a doctor who specialises in bowel cancers
- gynaecological surgeon – a doctor who treats problems with the female reproductive system
- plastic surgeon – a doctor who does operations (surgery) to repair or reconstruct tissue and skin.

Your nurse may ask you to prepare for your operation by doing things such as:

- taking laxatives to ensure your bowel is empty
- not eating or drinking for a certain time before the operation.

The operation

Pelvic exenteration usually takes about 8 hours, but it can take longer.

The surgery may be done in different ways:

- Open surgery – this means the surgeon makes 1 large cut in your tummy (abdomen).
- Laparoscopic (keyhole) surgery – this means the surgeon makes 4 or 5 small cuts in the tummy (abdomen) instead of 1 large cut. They pass a thin tube with a small camera (a laparoscope) into the tummy through 1 of the cuts. They then pass specially designed surgical tools through the other cuts to remove the cancer.

The surgeon starts the operation by checking for signs of cancer in other organs. They may take biopsies to send to a lab before they remove any organs. The biopsies will tell them if the cancer has spread outside the pelvis.

If there are no signs that the cancer has spread to other organs, the surgeon will continue with the operation.

If the cancer has spread the operation will be stopped. After surgery your cancer doctor and specialist nurse will talk to you about other ways of trying to control the cancer and help with any symptoms. This can be difficult to cope with especially as you will have prepared for the operation and what will happen after.

After pelvic exenteration

After the operation you will be in an intensive care or a high-dependency unit for the first few days. You will stay in hospital for a few days to a few weeks, depending on your recovery.

When you wake up after the operation, you will have dressings on your tummy (abdomen). You may also have dressings on the area between the genitals and back passage. If you have had reconstruction, you may have wounds and dressings on other parts of your body. Your nurse will tell you what to expect and give you advice about how to cope with your dressings and wounds.

You may also have the following:

A drip going into a vein (intravenous infusion)



This is used to give you food and fluids until you can eat and drink again. It may also be used to give you medications such as painkillers and antibiotics.

A fine tube going into your back (epidural)

This is used to give you painkillers.

A fine tube that passes down your nose and into your stomach (nasogastric tube)

This is to allow any fluids in the stomach to be removed, to stop you feeling sick.

One or more drainage tubes from your wound

These drain fluid or blood and are removed when the amount of fluid has reduced enough.

Bags covering a colostomy or urostomy

This will depend on what type of surgery you have. After a few days, you will be able to start to care for your own stoma and change the bag. Your stoma nurse will give you information and support to help you adjust. We have more information about having a [stoma](#) (colostomy or ileostomy) and having a [urostomy](#).

Urinary catheter

You may have a tube called a catheter to drain urine from the bladder into a bag.

Controlling pain

You will have painkillers when you first wake up from surgery. These will be given either:

- into a vein (intravenously)
- through a tube into the space around your spinal cord (epidural).

The painkillers are usually given continuously through a pump. The pump may have a hand control with a button you can press. This is called patient-controlled analgesia (PCA).

You press the button when you need more painkillers. You can press the button whenever you need it. It is designed so that you cannot give yourself too much. It is important to let the doctors or nurses know if you are still in pain, or if you feel any side effects from the painkillers. They can increase the dose or change the painkillers.

You will have painkillers for a few weeks after the operation. When you are ready to go home, your pain will be controlled with tablets. A nurse will give you tablets to take home.

We have more information about [managing pain](#) and [painkillers](#).

Starting to move around

After your operation, you will be encouraged to start moving around. The day after your operation, your nurses or physiotherapists will help you get out of bed.

If you have had plastic surgery you may be advised not to sit at first. You will be able to stand and lie on your side in bed. Your nurse will be able to tell you how long to avoid sitting for.

It is important to do regular leg movements and deep breathing exercises when you are not as mobile as you are used to. A physiotherapist will show you how.

They will also show you gentle exercises and give you advice on how and when to start walking. You will find it easier to move around when all your drips and drains have been removed.

Going home

The hospital nurses will tell you who will help you with your wound when you go home. You will also get support from a stoma nurse.

You will have injections every day for about 4 weeks. This is to thin the blood, which reduces the risk of [blood clots](#).

The hospital nurses will show you how to inject yourself, so you can do them at home. If you are not able to inject yourself, they will show a partner, family member or friend how to do it. Or they may be able to arrange a district nurse to do it for you.

It is important to try and move around as much as you can, and to keep doing the exercises you were shown in the hospital. This helps with your recovery.

If you have any pain when you are moving or doing your exercises, speak to your doctor or nurse. They may be able to adjust your dose of painkillers or change them to one that works better for you.

You may be given special equipment to take home with you. For example, you may be given an air cushion. Only use equipment that your doctor, nurse, physiotherapist or occupational therapist advises.

Recovery from pelvic exenteration

Your recovery from pelvic exenteration is individual and depends on the type of pelvic exenteration you had. It can take anywhere from a few weeks to a year. Your doctor or nurse will talk to you about what to expect. Build up your activities slowly and only do as much as you feel able to. As time goes on, you will start to feel stronger and have more energy. Talk to your surgeon or specialist nurse if you have any more questions about recovery.

You may want to know when you can:

- go back to work
- drive
- do housework
- get back to sports or other activities.

You will also be recovering emotionally, as well as physically, from the operation. Pelvic exenteration is a major surgery that can change your body and how you feel about your body. This takes time to deal with.

It is important to talk to someone about how you are feeling. This can be a nurse, counsellor, friend, family member or partner. Anyone you feel comfortable with.

About our information

References



Below is a sample of the sources used in our pelvic exenteration information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

Association of Coloproctology of Great Britain & Ireland (ACPGBI). Guidelines for the Management of Cancer of the Colon, Rectum and Anus. 2017. Available from www.onlinelibrary.wiley.com/toc/14631318/2017/19/S1 (accessed November 2022).

The European Society for Medical Oncology. Cervical cancer: ESMO Clinical Practice Guidelines for diagnosis, treatment and follow-up. 2017. Available from [www.annalsofoncology.org/article/S0923-7534\(19\)42148-0/pdf](http://www.annalsofoncology.org/article/S0923-7534(19)42148-0/pdf) (accessed November 2022).

Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Chief Medical Editor, [Professor Tim Iveson](#), Consultant Medical Oncologist.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

Date reviewed

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help